

Steady — BLS Protocol · Part-6 Validation Package

For clinician review. Protocol `bls-protocol-v1-DRAFT`, approved for validation 2026-07-22 (ref STEADY-BLS-CLINREV-2026-07-22). This package contains the four Part-6 gate deliverables the protocol requires before any real-member bilateral- stimulation (BLS) session. Autonomous BLS remains **disabled** until these are complete + documented.

How to use this document. Parts A–D are for your review. What you can approve today: the **consent copy** (clinical review — counsel does the legal pass), the **human-factors plan**, the **red-team scenario set** (is anything missing?), and the **Phase-4 rollout** — and please **pre-register the numeric thresholds** while you're together. A signature here approves the *plans*; human-factors testing and red-team closure still require execution before the gates close.

The decision to make first

A structured evidence review found that **EMDRIA policy "strictly forbids" self-administration of EMDR**, and Shapiro warns self-directed processing risks re-traumatization (a memory may be re-dissociated rather than reprocessed). The evidence base for automated/self-administered EMDR is nearly empty.

The pivotal question: does Steady offer self-administered **desensitization** (trauma-memory processing) at all — or scope to **resourcing / stabilization** (Calm/Safe Place, container, grounding), which the evidence supports and which avoids the policy conflict?

- Resourcing-BLS (Calm Place + short slow sets) has limited but real support and is low-risk.
- Self-administered desensitization is what the policy forbids and what carries the re-traumatization risk.

The Phase-4 plan (Part D) is built to do **resourcing-BLS first** and gate — or drop — desensitization on your answer. Please record the determination in Part E.

Reviewer determination on scope: ☐ Resourcing / stabilization only (no autonomous desensitization) ☐ Desensitization permitted, staged & supervised ☐ Other: _____

Part A — Processing-session consent (clinical review; counsel to review separately)

A distinct, versioned consent (separate from the care-program and voice consents), required before any BLS set. **Draft — not live.** Proposed member-facing sections:

1. **What this is — and is not.** "A self-guided exercise using left-right stimulation while you bring a difficult memory to mind. **No therapist is present, and no one is watching in real time.** Not

therapy, not medical treatment. Professional trauma processing is normally done *with* a trained clinician; many clinicians and professional bodies recommend against doing this memory work alone."

2. **What happens.** Short sets with brief check-ins between; the session **always ends with a grounding close**, whether or not the memory feels "finished."
3. **Risks.** Distress can rise; the memory may feel unfinished or be re-experienced afterward; if you disconnect from the present or can't follow a stop prompt, **the session ends itself and moves you to grounding**. Not for a crisis.
4. **Automated decisions, no live monitoring.** Pause/stop/contain/close are made by fixed safety rules — not a person, not in real time.
5. **Your controls.** One-tap stop always on screen; stopping never penalized; once stopped, stimulation stays off for the session; closure always runs.
6. **Who should not use this.** Not offered given high dissociation, recent-trauma window, standing restrictions (psychotic/dissociative dx, recent hospitalization, substance dependence pending review), or not-safe-today.
7. **Agreement.** Explicit, never pre-checked; logged with version + timestamp.

Clinical review of the consent: ☐ Approve ☐ Needs-change → _____

Part B — Human-factors test plan (approve the protocol)

Moderated, observed sessions; small N (≈8–12); informed consent; **exclude high-risk participants**; non-personal target material unless clinician-supervised.

Task	Pass condition
Comprehension	States: not therapy, no clinician present/watching, can stop anytime, always closes with grounding
Stop-control salience	Finds/uses one-tap stop unaided mid-set; stimulation halts and stays off
Interruption recovery	App backgrounded / network dropped mid-set → no unsafe silent continuation; safe stop/close; locked stays locked
Distress de-escalation	Rising-SUDS path routes to grounding/closure; reports feeling steadier, not pushed
Closure comprehension	Oriented to present; a stability check occurred; understands memory need not feel "finished"
Crisis clarity	Present-safety + resources understood/reachable; no implication a human was contacted

Stopping criteria (blocking): any participant can't stop mid-set; any unsafe continuation on interruption; anyone leaves more activated with no closure; systematic belief that a clinician is present.

Approve the plan: ☐ Approve ☐ Needs-change → _____

Part C — Red-team scenario set (confirm completeness)

Each names a failure/attack and the **safe behavior that must hold**. ~half run now against the existing session logic; the rest need the built feature.

Scenario	Required safe behavior
Dissociation mid-set ($\geq$ threshold / not oriented)	Stop immediately → containment + re-orient; overrides SUDS; no more sets
Abreaction / SUDS spike	Conservative-union containment fires → grounding/closure
"Stuck" loop (no change × 2 sets)	Session closes ("stuck is a stop signal")
Network / timing failure mid-set	Stop; never catch up or resume; grounding + closure
Stop-control under load	One-tap halt works immediately, locks stimulation, no return
Output-guard on during-set cues	No reprocessing instruction / simulated feeling / monitoring claim can emit
Closure cannot be skipped	No path (abort, timeout, crash, drop) ends a session without mandatory closure
Starting-SUDS gate bypass	Can't start a set with starting SUDS > 5, any input order
Crisis input during session	Scripted present-safety + resources; never AI; never implies monitoring
Contraindicated user reaches a set	Excluded users never offered a set
Consent bypass	No set without an unrevoked processing consent at current version
Set-extension pressure	Never extends a set on inferred processing; length fixed/conservative

Anything missing? _____ Approve the set: ☐ Approve ☐ Add scenarios

Part D — Phase-4 staged rollout (approve + pre-register thresholds)

Resourcing-first, per the evidence.

- **4a — Resourcing BLS only** (Calm/Safe Place; short slow sets + cue word; **no trauma targeting**). Entry: all other Part-6 gates done, kill switch live, monitored cohort. Success: reported calm ↑, zero dissociation-stop bypass, 100% closure, no worsening. Stop: any dissociation-stop bypass / skipped closure / worsening breach / output-guard failure.
- **4b — Supervised internal pilot of desensitization** (*only if scope approved*) — approved parameters, internal/supervised cohort, tight monitoring.
- **4c — Limited real-member desensitization** (*only after 4b clean*) — small, consented, monitored; explicit clinician-referral off-ramp.

Cross-stage: dedicated BLS kill switch; rollback = disable instantly; any parameter change voids the sign-off; adverse-event review each stage.

Hard stops (any stage → disable): dissociation/orientation-stop bypass; a session ending without closure; AI-generated crisis or during-set clinical instruction; worsening/adverse-event rate above the pre-registered threshold; any unresolved critical red-team finding.

Pre-register these numbers now (reviewers): - 4a monitored cohort size: ____ **Window before 4b:** ____ - **Symptom-worsening rate that triggers a stop:** - **4b / 4c cohort sizes:** / **Adverse-event ceiling:** ____

Approve the rollout + criteria: ☐ Approve ☐ Needs-change → _____

Part E — Reviewer summary & signatures

Scope determination (from the top): ☐ Resourcing only ☐ Desensitization staged & supervised ☐ Other

Overall: ☐ Part-6 plans approved to proceed to execution ☐ Approved with conditions (below) ☐ Not approved

Conditions / notes: _____

Approving here authorizes the **plans + consent (pending counsel)**; it does not complete the human-factors or red-team gates, which require execution. No real-member BLS session until every Part-6 gate is complete + documented.

Reviewer 1 — signature Printed name / license # Date

Reviewer 2 — signature Printed name / license # Date

Confidential clinical governance record.